

Steady — Handoff 10 and 11: clinical questions and sign-off

Reference STEADY-CLINREV-2026-09-25-10F-v2 · Prepared 2026-09-25 · Follows STEADY-CLINREV-2026-09-24-10 · Generated from the app's registers by scripts/gen-h10-followup-worksheet.ts

Three parts, for three different people. Part A is for the two psychologists who signed the 24 September review. Part B is for evolvedMD's clinical lead (Handoff 11 names evolvedMD as the partner). Part C is the founder's. Mark one option per item, add notes, initial, and sign at the end of your part.

Part A — For Rebecca Altschuler, PhD, and John Allen, PhD

Follow-ups to the 24 September review (STEADY-CLINREV-2026-09-24-10). Each item says what the app does today; mark one option or write your own. Nothing here changes a row you already signed unless you say so.

A1. Open questions

1. clinical.itq-between-visits

The second trauma questionnaire (the International Trauma Questionnaire) has no published past-week version that we know of. How often should members take it between visits?

What happens now: It is given once every thirty days, with its standard past-month wording.

- ☐ **Monthly, as written** recommended
Keep the published wording and give it once a month, next to the weekly checklist. *Then:* Nothing changes.
- ☐ **Weekly, with an adapted past-week wording**
Change its wording to ask about the past week. It would no longer be the published questionnaire, and its scores could not be compared with published ones. *Then:* The psychologists write and sign the adapted wording, then it opens every seven days.
- ☐ **Only at the start**
Ask it once when someone joins and leave between-visit tracking to the checklist. *Then:* It leaves the between-visit questionnaire list.
- ☐ **Something else** _____

Notes:

Initials _____

2. clinical.program-fit-pause-length

After the first eight questions stop someone because of thoughts of self-harm in the past month, or an unsafe situation now, how long before they may answer again?

What happens now: Twenty-four hours, as before. Answers a person cannot truthfully change — a diagnosis, a hospital stay in the past year, relying on substances, being under eighteen — now stay in place until someone from the care team has reviewed them and written down what they did; answering again no longer lifts them.

☐ **Until the question's own period has passed** recommended

The self-harm question asks about the past thirty days, so an honest answer cannot change for thirty days. A person from the care team can reopen it sooner after talking with them. *Then:* The pause becomes thirty days for that question, and stays twenty-four hours for an unsafe situation now.

☐ **Fourteen days**

The figure in the clinical rules document the safety rules were built from. *Then:* The pause becomes fourteen days for both questions.

☐ **Keep twenty-four hours**

A short pause, relying on honest answers the second time. *Then:* Nothing changes.

☐ **Something else** _____

Notes:

Initials _____

3. clinical.steadier-sleep-gates

The sleep program's four parts came with no level of distress or safety at which each one opens. The second part asks a member to get out of bed at night when they cannot sleep. Which levels should the parts use?

What happens now: The levels you already signed for the two night practices (row A14), chosen by the product owner on 25 September: parts one, three and four open on any day, even the hardest, like "After a bad dream"; part two opens only on a steadier day with distress at 7 or below, like "Back to rest". On other days it says "not today" and nothing is lost.

- ☐ **Keep the night-practice levels** recommended
What runs now. The part that asks a member to get up at night waits for a steadier day. *Then:* Nothing changes.
- ☐ **Open every part on any day**
Getting up after twenty minutes awake is safe enough to suggest even on the hardest days. *Then:* Part two opens at the same level as the others.
- ☐ **Hold the whole program for steadier days**
Treat it like the activity program, which waits for a steadier day throughout. *Then:* Every part needs a steadier day, with distress at 7 or below.
- ☐ **Something else** _____

Notes:

Initials _____

4. product.feelings-word-list

Two signed screens ask a member to pick from a feelings word list — Feeling and Relating's "Pick two words for how you feel right now", and the thought record's "What did you feel?" — but the signed content does not include a list. Should there be one, and which words?

What happens now: Members type one or two words of their own. Nothing is suggested, so nobody is steered toward a word that does not fit; the cost is that a member who cannot find a word gets no help finding one.

- ☐ **Add a short list the psychologists write** recommended
A plain list, including mixed and body-based words, reviewed like the rest of the pack. *Then:* The list appears once its review row is signed, with typing still allowed.
- ☐ **Keep typing only**
The member's own words, always. No list to steer them. *Then:* Nothing changes.
- ☐ **Something else** _____

Notes:

Initials _____

5. clinical.riding-strong-feelings-gates

The signed row says **Riding Strong Feelings** is gated "per underlying skills" without giving levels. Each part holds skills with different levels. When should a part open?

What happens now: A part opens when any of its skills would, and inside it each skill still shows only when its own level allows. So on a very hard day, parts one and two open with "Move it out" and "Find the room" — the skills made for that — and parts three and four wait for a steadier day.

☐ **When any of its skills would** recommended

What runs now. The program is there when feelings run high, showing only what fits the day. *Then:* Nothing changes.

☐ **Only when all of its skills would**

A part opens only when every skill in it is open, so the part is whole or not there. *Then:* Parts one and two wait for a steadier day, with distress at 7 or below.

☐ **Something else** _____

Notes:

Initials _____

6. clinical.thought-record-privacy

Members' thought records are signed as "private to the member". The strength of the feeling, before and after, is a number the care team could use to see whether the skill helps. Should those two numbers reach the care team, without the words?

What happens now: Nothing about a thought record reaches the care team: not the words, not the numbers, not that one was written. The program activities are different: their ratings do reach the care team's analytics.

☐ **Keep them fully private** recommended

The signed row read strictly: a thought record is the member's alone, numbers included. *Then:* Nothing changes.

☐ **Share the two numbers only**

The care team sees that a record was made and how strong the feeling was before and after, never the words. *Then:* The two numbers join the care team's analytics, the way program ratings do.

☐ **Something else** _____

Notes:

Initials _____

A2. Decisions the product owner made on 25 September — please confirm

7. clinical.sleep-entry-screen-retake answered by the product owner — please confirm

The sleep program asks three safety questions on joining; a yes to any leaves out the part about getting up at night. If someone leaves the program and joins again, should the questions be asked again?

Decided: Yes, after leaving and rejoining — the product owner's decision on 25 September, with the risk that a member could answer no the second time to get the left-out part back. For the psychologists to confirm at the next review.

☐ **Yes, after leaving and rejoining**

Things change: a health question can stop applying. Leaving clears the answers and joining again asks all three again. The risk: someone could answer no the second time only to get the left-out part back. *Then:* The earlier answer is kept, marked cleared, so the care team can see a change. A different answer the second time brings the part back.

☐ **No, the first answer stands**

Once a part is left out it stays out, unless someone from the care team reopens it. *Then:* Joining again skips the questions and keeps the part left out.

☐ **Something else** _____

Notes:

Initials _____

8. clinical.complex-path-program-before-review answered by the product owner — please confirm

Feeling and Relating is signed for the complex trauma readiness path only once that path's clinician review is done, but the app cannot record that review yet. What should members on that path see meanwhile?

Decided: Hide it until reviews exist — the product owner's decision on 25 September.

☐ **Hide it until reviews exist** recommended

Members on the complex trauma path do not see it, even if they are on the trauma path as well. Recording a review becomes its own piece of work. *Then:* Members on the trauma path alone see it; the complex path waits.

☐ **Build the review mark now**

A care-team action on the person record that opens the program for that member. *Then:* The complex path sees it once someone from the care team records the review.

☐ **Something else** _____

Notes:

Initials _____

A3. Wording and readings to confirm

9. Row KB_SELF_HOLD_REPLACES_BUTTERFLY (unsigned)

The self-hold that replaced the Butterfly Hug is confirmed by the Handoff 10 review (CV10_A01, 2026-09-24). This row covers what that row did not: made at the same time and for the same reason, two movement practices were reworded — the neck roll ('roll it slowly side to side') now rests the chin for a few breaths, and the rooting stance's 'sway forward and back, side to side' now settles weight into the heels and then evenly across both feet — and the calm-place lesson no longer says to pair the calm place with bilateral stimulation. Confirm these wordings, or mark needs-change.

☐ Approve

☐ Needs change (say what)

Notes:

Initials _____

10. "Marching in place" in the skill "Move it out" (S04, signed as A02/A03)

Step 1 reads: "Pick something your body can do safely: a brisk walk, marching in place, pushing against a wall." Marching alternates left and right in a steady rhythm, which is close to the bilateral shape v1 excludes. It was signed as written and ships as written. Should it stay, or be replaced (for example with "standing up and stretching")?

☐ Keep as written

☐ Replace (write the wording)

Notes:

Initials _____

11. How the 24 September sheet was read (1 of 3)

CV10_B02 asked the reviewers to choose the proposal, the knowledge-base default, or their own values, and was approved without a choice marked. It is read as approving the proposal, the only values written in the row: units 1 and 2 at stabilization, ceiling 6; units 3 and 4 at cautious, ceiling 6; Gentle items only at stabilization.

☐ That is what we meant

☐ We meant something else (say what)

Notes:

Initials _____

12. How the 24 September sheet was read (2 of 3)

CV10_A04 asked whether the companion should keep offering the cold-water skill at all, and was approved without an answer to that part. The skill stays out of the member library and the companion's behaviour is unchanged, which is the row's own default.

- ☐ That is what we meant
- ☐ We meant something else (say what)

Notes:

Initials _____

13. How the 24 September sheet was read (3 of 3)

CV10_C05 approves PROMIS Sleep Disturbance 8a as the candidate measure. No sleep measure is added until its licensed item text is supplied: the content pack does not contain it and it is not written from memory.

- ☐ That is what we meant
- ☐ We meant something else (say what)

Notes:

Initials _____

A4. Handoff 11 (the evolvedMD evaluation build): rows for you

Handoff 11 names you as reviewers of these three. None is built yet; each says what will be.

14. CV11_02

Member-controlled sharing of entries (W4): on any entry (activity reflection, thought record) a member can tap "Share with my care team". Only shared entries are visible to the care manager; sharing is per entry and revocable; the default is private. Confirm, noting it changes CV10_D04's "entries private to the member" into private unless the member shares.

What is built: Not built yet (Handoff 11 package 3). Until this row is approved, nothing a member writes reaches the care team, which is today's rule.

- ☐ Approve
- ☐ Approve with changes (write them below)
- ☐ Needs change

Notes:

Initials _____

15. CV11_03

Problem-solving module content (W11): a short structured program Steady drafts (define the problem, set a realistic goal, list options, weigh them, choose one, plan steps, review), assignable by care managers and self-startable at the stabilization tier. Approve the text, or the partner loads its own worksheets instead.

What is built: Not written yet (package 11). Steady will draft it for this row; it stays in the demo and evaluation tenant, marked pending review, until approved.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

16. CV11_05

Visit prep member prompt (W5): an optional "what I want to talk about" prompt sent to the member 2 days before a visit, shown to the care manager on the visit prep summary. Approve the prompt and its timing.

What is built: Not built yet (package 6). The prompt's wording will be added to this row before it is shown to anyone.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

Part A sign-off

Rebecca Altschuler, PhD (AZ PSY-005804) _____ Date _____

John Allen, PhD (AZ PSY-002055) _____ Date _____

Part B — For evolvedMD's clinical lead (Lane E, and Handoff 11)

The clinician-assigned lane was built ahead of its gate on the product owner's instruction (25 September) so a team can review and test it. It is not live: outside the demo it does not exist until these rows are approved. This part is for the clinical lead of the provider partner who will assign and review, not for Part A's reviewers.

B1. Rows

17. CV10_E01

Lane rules: never self-startable; active assignment required; assignments expire; distress 0 to 10 before and after; grounding, SOS, and clinician flag if after exceeds before by 3 or more, or exceeds 7.

What is built: Built and testable in the demo only. A practice is listed only for a person with a live assignment, which must have an end date; an expired or guessed link says "not available". Each start asks the existing safety engine (steady tier, no crisis today, no dissociation hold). Distress 0 to 10 before and after; a rise of 3 or more, or above 7, shows grounding and SOS and alerts the care team. Stop is on every step and is never counted against anyone.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

18. CV10_E02

Written exposure container: narrative stored encrypted, visible to member and assigning clinician only, fully excluded from the companion. Retention per partner policy.

What is built: "Written exposure (clinician-assigned)", about 30 minutes. No protocol text: the partner supplies it; the demo shows placeholders. Writing is encrypted, readable by the person and the assigning clinician only, and unreachable from the companion and every AI path (tested).

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

19. CV10_E03

CPT worksheet container: worksheets supplied by the partner under license; same lane rules.

What is built: "Worksheets (clinician-assigned)", about 20 minutes. Worksheet boxes the partner defines; the same handling as E02.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

20. CV10_E04

Imagery rehearsal container (nightmares): supplied by the partner; same lane rules and narrative handling as E02.

What is built: "Nightmare rehearsal (clinician-assigned)", about 20 minutes. The same handling as E02; no content until supplied.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

21. CV10_E05

Protocol source and fidelity: who supplies each protocol, under what license, and who is responsible for fidelity. No protocol content is written by Steady.

What is built: Nothing is written by Steady. Each practice records its content owner as the partner and cannot run outside the demo until the partner's licensed content is loaded.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

B2. Handoff 11 rows

22. CV11_01

Triage rules and thresholds (W2): the care manager's queue sorts by needs-attention in this order: PHQ-9 item 9 positive since last review; crisis script shown since last review; distress after an assigned activity above threshold; not at target (less than 50% PHQ-9 reduction at or after week 10); measure overdue beyond cadence plus 7 days; no app activity in 14 days after being active; visit in the next 2 days. Confirm the rules, order and thresholds.

What is built: Not built yet (Handoff 11 package 4). The rules are written into this row as the handoff gives them; confirm or change them before they sort anyone's queue.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

23. CV11_04

Escalation mapping and member crisis copy (W9): the member crisis path is unchanged (scripted interrupt, 988, SOS) with the partner's care-team contact and hours added; item-9 positives and crisis events become a top-priority queue item for the assigned care manager and appear on the leadership safety view; no response time is ever promised to members.

What is built: Not built yet (package 8). Today the member's crisis path is 988 and SOS; no care-team contact is shown until you supply one (question below).

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

B3. Questions

24. partner.crisis-check-on-exposure-writing

For the partner's clinical lead: every piece of writing in the app is checked for crisis language before it is saved, and a match saves nothing and opens the crisis page. Exposure writing can describe past harm in words that match. Should the check run on it, and what should happen on a match?

What happens now: The check runs. A match saves none of that step's writing, ends the practice, alerts the care team without the words, and opens the crisis page. Earlier steps' writing stays saved.

- ☐ **Check it, keep the writing, flag the clinician** recommended
A match still opens the crisis page and alerts the care team, but the writing is kept for the assigning clinician. *Then:* Nothing written is lost; the clinician sees it with the flag.
- ☐ **Check it and save nothing, as now**
The same rule as everywhere else in the app. *Then:* Nothing changes.
- ☐ **Do not check exposure writing**
The assigning clinician reviews it; the distress ratings and flag still apply. *Then:* The check is skipped for writing steps in this lane only.
- ☐ **Something else** _____

Notes:

Initials _____

25. partner.who-reads-assigned-writing

For the partner's clinical lead: what a person writes in a clinician-assigned practice is readable by them and by the clinician who assigned it. Should anyone else on the care team be able to read it?

What happens now: Only the person and the assigning clinician. A colleague who opens it is told whose it is and shown nothing; the distress flag still reaches the care team's queue, without the words.

- ☐ **Assigning clinician only** recommended
As the handoff says: the person and the one clinician who gave them the practice. *Then:* Nothing changes.
- ☐ **Their whole care team**
Anyone caring for the person can read it, for continuity when the assigning clinician is away. *Then:* Every clinician on the person's care team can open the writing; each read is still recorded.
- ☐ **Something else** _____

Notes:

Initials _____

26. partner.assigned-writing-retention

For the partner's clinical lead: how long is writing from a clinician-assigned practice kept, and may the person delete it themselves?

What happens now: Kept until the person's account is deleted. The person cannot delete it on their own, because it may form part of the partner's clinical record; the handoff leaves this to the partner's record policy.

☐ **The partner's record policy** recommended

Kept for the period the partner's records must be kept, then removed. *Then:* A retention period is set for this lane from the partner's policy.

☐ **The person can delete it**

Like everything else they write in Steady, with two taps. *Then:* A delete control appears beside their writing; the clinician's view shows it was deleted.

☐ **Something else** _____

Notes:

Initials _____

27. partner.distress-flag-levels

For the partner's clinical lead: the signed lane rule flags the assigning clinician when distress after a practice is 3 or more above distress before, or above 7. Are those the right levels, and should a practice the person stopped be flagged by the same rule?

What happens now: Those levels, applied whether the practice was finished or stopped, when a rating after was given. A person who stops may skip the rating, and then nothing is flagged.

☐ **Keep them, finished or stopped** recommended

The rule as written, applied to every run with a rating. *Then:* Nothing changes.

☐ **Also flag any stop**

A stop is not a failure, but the clinician may want to know it happened. *Then:* Every stopped practice sends a gentle note to the clinician's queue.

☐ **Different levels**

Name the rise, and the level after the practice, that should reach the clinician. *Then:* The two levels change.

☐ **Something else** _____

Notes:

Initials _____

28. partner.lane-titles-and-lengths

For the partner's clinical lead: the three practices need names a person reads and a length. The handoff names one, "Written exposure (clinician-assigned)". Are "Worksheets (clinician-assigned)" and "Nightmare rehearsal (clinician-assigned)", at about 30, 20 and 20 minutes, right?

What happens now: Those names and lengths, marked as drafts; no method is named to the person.

☐ **Keep them** recommended

Plain names that describe the activity without naming a method. *Then:* Nothing changes.

☐ **The partner's own names**

Use whatever the partner's protocols call them for patients. *Then:* The names and lengths change to the partner's.

☐ **Something else** _____

Notes:

Initials _____

29. partner.registry-and-export

For the partner: which registry and EHR systems do your teams use, and which columns should the PHQ-9 and GAD-7 export have?

What happens now: The export is a file in the evaluation (CSV, and FHIR Observations coded with LOINC 44261-6 and 70274-6), in a generic column layout until the partner's is known.

☐ **Send your column layout** recommended

The CSV matches what your registry imports, column for column. *Then:* The export's columns change to yours.

☐ **Use the generic layout**

One row per measure: patient reference, instrument, date, total score, change from baseline. *Then:* Nothing changes.

☐ **Something else** _____

Notes:

Initials _____

30. partner.measure-cadence

For the partner: how often should members complete the PHQ-9 and GAD-7 between visits?

What happens now: Every 14 days, and before each visit, as the handoff proposes.

☐ **Every 14 days and before visits** recommended

The handoff's proposal: often enough to see a trend, not so often it becomes a chore. *Then:* Nothing changes.

☐ **Another cadence**

Name the number of days, and whether to add one before each visit. *Then:* The cadence in the tenant's settings changes.

☐ **Something else** _____

Notes:

Initials _____

31. partner.treat-to-target

For the partner: which treat-to-target thresholds do your consultants use?

What happens now: Response is a 50% drop in PHQ-9; remission is PHQ-9 below 5; a patient not responding by week 10 goes to consultant review.

☐ **Keep those thresholds** recommended

The collaborative care defaults the handoff proposes. *Then:* Nothing changes.

☐ **Your own thresholds**

Name the response drop, the remission score and the review week you use. *Then:* The thresholds in the tenant's settings change.

☐ **Something else** _____

Notes:

Initials _____

32. partner.care-team-contact

For the partner: what care-team contact and hours should members see, and who should receive alerts?

What happens now: Members see 988 and SOS only; no care-team contact is shown until you supply one. In the evaluation, alerts go to a test inbox, never to a real phone.

☐ **Supply a contact and hours** recommended

A number and the hours it is answered, shown beside 988, never with a promised response time. *Then:* Members see your contact; alerts route to the people you name, once live.

☐ **Show 988 and SOS only**

No care-team contact is shown to members. *Then:* Nothing changes.

☐ **Something else** _____

Notes:

Initials _____

33. partner.most-used-interventions

For the partner: which interventions do your behavioral health managers use most, and do you have worksheets to load?

What happens now: Assignable today: Steady's signed skills, programs and lessons, and member thought records; a problem-solving module is drafted for your review, and your own worksheets can be loaded into protocol slots.

☐ **Tell us your top interventions** recommended

Behavioral activation, problem solving, CBT skills or others, in the order your teams use them. *Then:* The assign menu is ordered to match, and your worksheets are loaded into slots.

☐ **Use Steady's catalog as it is**

The signed content, in its current order. *Then:* Nothing changes.

☐ **Something else** _____

Notes:

Initials _____

34. partner.pcp-view

For the partner: do you want primary care providers to have a view of their own patients in Steady at all?

What happens now: The primary care role exists and sees nothing yet; the monthly summary is built later in this build.

☐ **Yes, the monthly summary** recommended

Each patient's engagement, measure direction and any open consultant recommendation, and nothing else. *Then:* The summary is built as the handoff describes.

☐ **No primary care view**

Primary care providers do not get accounts. *Then:* The role and its page are removed from the evaluation tenant.

☐ **Something else** _____

Notes:

Initials _____

35. partner.pilot-sites

For the partner: which sites or states would the eventual pilot use, and are the virtual Northeast sites the better first fit?

What happens now: The evaluation seeds generic site names across AZ, MA, NH and ME; nothing depends on the answer until a pilot is planned.

☐ **Virtual Northeast sites first** recommended

Largely virtual care, which fits a between-visit app most naturally. *Then:* Pilot planning starts with MA, NH and ME virtual sites.

☐ **Name the sites**

Tell us which sites and states you would start with. *Then:* Pilot planning starts there.

☐ **Something else** _____

Notes:

Initials _____

Part B sign-off

Partner clinical lead (name, credentials, licence) _____ Date _____

Part C — For you (founder, with counsel): not clinical

Items no clinician can sign. Tick when done and file the document where noted.

36. F01 — sleep measure licence

Sleep measure licensing: confirm the terms for using the chosen sleep measure inside a commercial product. You signed F01; the licence itself is not in the repository. Until its item text is supplied, the sleep program adds no measure (C05).

☐ Licence filed (where)

☐ Not yet

Notes:

Initials _____

37. F04 — audio rights

Audio rights: written rights for narrator recordings and soundscapes. You signed F04; the agreements are not in the repository, and no recordings exist yet. Package 11 (audio) waits on them.

☐ Agreements filed (where)

☐ Not yet

Notes:

Initials _____

38. Phase 3 — before any real person uses the clinician-assigned lane

Handoff 10 §6 lists four conditions and the lane was built before them on your instruction: (1) Handoff 03's activation gate, including the repo delta note and Handoff 02's scheduler; (2) a signed provider partner whose clinicians assign and review; (3) licensed protocol content from that partner; (4) HIPAA posture, a BAA, and the Postgres cutover. Part B's sign-off is also required.

☐ Acknowledged: none of the four is met today

☐ Some are met (say which)

Notes:

Initials _____

Part C sign-off

Founder _____ Date _____